جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : كلير نبيل فوزي مسعود

المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** 1 ش يعقوب عطاالله ش الهنيدي الهرم

رقم البطاقة : 28609292101465 **رقم التليفون :** 01221807622 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 29-09-1986 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: كلير نبيل فوزي مسعود
National ID: 28609292101465

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



كلير

نبيل فوزى مسعود

١ ش يعقوب عطاالله الهندى

الهرم - الجيزه

٢٨٦٠٩٢٩٢١٠١٤٦٥

١٩٨٦/٠٩/٢٩

JZ1010028

National ID · Front
[[DOCTYPE:ID_FRONT]]

CamScanner



٢٨٦٠٩٢٩٢١٠١٤٦٥ ٢٠٢٣/٠٤

حاصله على ليسانس اداب

أنثى مسيحية متزوجة

بيتر وحيد وديع توفيق

البطاقة سارية حتى ٢٠٣٠/٠٤/٢٩

١



National ID - Back
[[DOCTYPE:ID_BACK]]

CamScanner

Name: كلير نبيل فوزي	
MR. : 839	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 39-year-old with a history of left breast cancer, classified as CT3, no metastasis, ER 8/8, PR 6/8, HER2 SISH amplified, and REC 4 AC 12. The patient underwent a left mastectomy with axillary lymph node dissection (YPTIS YPNO0/18) and postoperative radiation therapy to the chest wall and regional nodes. Adjuvant chemotherapy consisted of 12 cycles of Taxol and Phesgo, initiated on 14/06/2025. The patient also received adjuvant endocrine therapy with Arimidex (private) and Zoladex, with the first Zoladex dose administered on 13/02/2026. A new echocardiogram was requested following the completion of the 13th cycle of Phesgo.

The patient continued endocrine therapy with Arimidex and Zoladex, with the 14th cycle of Phesgo scheduled on 13/02/2026. The 15th and 16th cycles of Phesgo were planned, with Zoladex pharmacy due date on 14/03/2026. Clinical pharmacy visits documented the patient receiving the 15th cycle of Phesgo with Arimidex (EF 60% on 13/02/2026) and the 16th cycle of Phesgo with Arimidex plus Zoladex (EF 60% on 13/02/2026). A subsequent pharmacy visit on 08/04/2026 confirmed the patient was on the 16th cycle of Phesgo with Arimidex (EF 60% on 13/02/2026).

An echocardiogram performed on 24/04/2026 showed an ejection fraction of 65%, and the 17th cycle of Phesgo with Arimidex was considered if approved, with supportive Zoladex to be started on 06/06/2026.

The most recent internal medicine note on 29/08/2026 reiterates the patient's breast cancer history and notes that the patient received Zoladex 10.8 mg subcutaneously today, along with supportive therapy.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 10/7/2025 G

Name	كلير نبيل فوزى	MRN	1000/2025
		ID	٢٨٦٠٩٠٩٠١٠١٤٦
Age	38	Phone	01221448070
Menopausal status	Chemotherapy induced amenorrhea	Marital status	Married
NRS SCORE		Family history	- ve
BMI		REASON OF MDT	Post op
CO morbidity	HTN		

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	Ki67:
818	618	Amplified	

Sonomammography:

Mets Workup:

R L

TNM

Recommendations: Post NACT did operation outside hospital Lt Skin reducing mastectomy + ALND + expander

MDT OUTCOME & PLAN ① Thos go to Centuie one year ② Zdarlex + TAM

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form [[DOCTYPE:MDT_FORM]] CamScanner

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	: الأستاذه / كلير نبيل فوزى مسعود	Request No.	: 13003225
Patient No.	: 28609292101465	Request Date	: 28/06/2025
Path No.	: SR-874/25	Verification Date	: 30/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data. Left breast cancer, Post-neoadjuvant chemotherapy.

Nature of Specimen: Slides Revision (Left skin reducing mastectomy and axillary clearance).

Gross Examination:
- Sixteen referred paraffin blocks and sixteen referred slides were received labeled by (patient name) and (962532).

Microscopic Examination:
- No residual viable remnants of the previously diagnosed invasive tumor.
- DCIS: Tiny focus of intermediate grade DCIS (Solid pattern with extensive calcification).
- Angioinvasion: Not detected.
- Perineural invasion: Not detected.
- Skin: Free.
- Lymph nodes: 15 lymph nodes free of tumor deposits and showing no therapy effect (0/15).

Diagnosis:
Left breast cancer (Post-neoadjuvant chemotherapy), Slides revision (Left skin reducing mastectomy & axillary clearance):
- No residual invasive foci.
- A tiny focus of intermediate grade (DCIS) (Solid pattern with extensive calcifications).
- Skin: Free.
- Lymph nodes: 15 lymph nodes free of tumor deposits, (0/15).
- Stage: ypTis, ypN0.

Signature :
Prof. Dr. Maha Guimei.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY REPORT]]
CamScanner

MRN:	10124252704
Patient Name:	KALER NABIL FAUZI MASOUD
D.O.B:	29/09/1986
IC No.:	28609292101465
Sex:	Female
Location:	External Clinic

MOLECULAR GENETICS

Sish

Specimen Id	2411160001	**Collection Date**	16/11/2024 07:27 AM
Accession No	SISH24-369	**Received Date**	16/11/2024 07:27 AM
Sample Type	Tissue	**Doctor**	Abd ALLAH Mohamed Gamil

Sish Report

pathology number
7014/24
Clinical Data
LT breast cancer.
Gross Picture :
Referred block submitted for HER 2 detection by silver in situ hybridization.
System Used :
Ventana BenchMark ULTRA autostainer.
Probe:
Ventana Inform HER2 Dual ISH DNA Probe Cocktail
Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations,2018 Twenty nuclei (each containing red (Chr17) and black (HER2) signals) should be enumerated. The final results for the HER2 status are reported based on the ratio formed by dividing the sum of HER2 signals for all 20 nuclei divided by the sum of Chromosome 17 signals for all 20 nuclei. The amplification status is defined as Amplified if the HER2/Chromosome 17 ratio >/=2.0 and the average Her2 gene copy number is >/=4.0. It is Non-Amplified if the HER2/Chromosome 17 ratio <2.0 with the Her2 gene copy number is <4.0.
If the HER2/Chr17 ratio is <2 and the Her2 gene copy number is between 4.0 and 6.0, or, HER2/Chr17 ratio is >/=2 and the Her2 gene copy number is <4, or HER2/Chr17 ratio is <2 and the Her2 gene copy number is >/= 6.0, an additional work should be done.
(Laterality) breast Cancer:
Left
Site 1
Diagnosis #1
Lt. breast Cancer:
Her-2 gene: Amplified with clustering (Her2/Chr17 >2 and average Her2 copy number is >4).
Final Result #1
Amplified
Site 2
Laterality #2
Signature
MAHITAB IBRAHIM MOHAMED
Co-Signed by:

Reported and electronically endorsed by
Mahitab Mohamed Elichamy Mohamed

CamScanner
MOLECULAR-GENETICS
[[DOCTYPE:OTHER_MOLECULAR_GENETICS]]

MRN:	10124252704
Patient Name:	KALER NABIL FAUZI MASOUD
D.O.B:	29/09/1986
IC No.:	28609292101465
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	2411070255	**Collection Date**	07/11/2024 03:16 PM
Accession No	IMM24-5313	**Received Date**	07/11/2024 03:17 PM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

7014/24

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 (4B5) rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(laterality) breast Cancer:

left

Her2 Result - 1

Equivocal [score 2+]. Recommended For SISH

Comment - 1

**SISH test for HER-2 gene copy number is ordered.

Laterality - 2

Signature

MAHITAB IBRAHIM MOHAMED PHD

Co-Signed by:

Reported and electronically endorsed by

Mahitab Mohamed Eltohamy Mohamed

CamScanner
Immunohistochemistry
[[DOCTYPE:IHC]]

MRN:	10124252704
Patient Name:	KALER NABIL FAUZI MASOUD
D.O.B:	29/09/1986
IC No.:	28609292101465
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2411070254	**Collection Date**	07/11/2024 03:16 PM
Accession No	IMM24-5312	**Received Date**	07/11/2024 03:17 PM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

7014/24

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for ER/PR Immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.
- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :

ER : positive, strong nuclear reaction in 100 % of tumor cells (8/8).
PR :positive, strong nuclear reaction in 15% of tumor cells (6/8).

Signature

MAHITAB IBRAHIM MOHAMED PHD

Reported and electronically endorsed by

Mahitab Mohamed Elsohamy Mohamed

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Immunohistochemistry
[[DOCTYPE:IHC]]

MRN:	10124252704
Patient Name:	KALER NABIL FAUZI MASOUD
D.O.B:	29/09/1986
IC No :	28609292101465
Sex:	Female
Location:	External Clinic

CYTOPATHOLOGY

(Cytology - 1 Site) فحص خلوي

Specimen Id	2411050239	**Collection Date**	05/11/2024 02:02 PM
Accession No	CYTO24-1416	**Received Date**	05/11/2024 02:02 PM
Sample Type	Fluid/ Smear	**Doctor**	Abd ALLAH Mohamed Gamil

Nature of Specimen:
U/S-guided FNAC from Lt. axillary Indeterminate L. node.
Gross Picture:
4 unstained smears were received, Pap-stained.
1 cell block block received, Hx&E-stained
Microscopic Picture:
Smears and cell block show reactive lymphocytes at different stages of maturation with scattered tangible body macrophages. No atypical or malignant cells seen.
Diagnosis:
Lt. axillary L. node, U/S-guided FNAC:
Consistent with reactive lymphoid cells, negative for malignancy.
Comments:
Recommended for clinic-radiologic correlation.
Signature

MONA MOHAMED MAMDOH

Reported and electronically endorsed by
Mona MOhamed Mamdouh Abdelfattah

CamScanner
CYTOLOGY
[[DOCTYPE:OTHER_CYTOLOGY]]

MRN:	10124252704
Patient Name:	KALER NABIL FAUZI MASOUD
D.O.B:	29/09/1986
IC No.:	28609292101465
Sex:	Female
Location:	External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2411050240	**Collection Date**	05/11/2024 02:03 PM
Accession No	SUR24-7014	**Received Date**	05/11/2024 02:03 PM
Sample Type	Tissue	**Doctor**	Abd ALLAH Mohamed Gamil

Clinical Information
LT breast highly suspicious lesion BIRADS4c.
Gross Picture
4 Cores 1-1.3 cm.
Microscopic Picture
A case of invasive duct carcinoma ,grade 3.There is an associated high grade DCIS component of solid pattern. Lymphovascular emboli were detected .Stroma is desmoplastic with mild inflammatory cell reaction.
Diagnostic Summary
Tumour type:
Invasive duct carcinoma ,grade 3
Laterality
Left
Specimen type:
Tru-cut biopsy.
Location
Histological grade
Mitoris score (1-3):
2
Tubular formation score (1-3):
3
Nuclear grade (1-3):
3
Histological grade (1-3):
3
Diagnosis
Left breast,TRU-cut biopsy:
Invasive duct carcinoma ,grade 3
Comments
ER, PR & HER-2 are ordered.
Biopsy reporting category
B5b= Invasive cancer
Signature
MAHITAB IBRAHIM MOHAMED PHD

Reported and electronically endorsed by
Mahitab Mohamed Ebrahimy Mohamed

CamScanner
Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : كلير نبيل فوزى
Analysis ID : 908512
Age : 38 Y : 1 M : 2 D
Gender : Female
Referred By Prof. Dr. : عماد برسوم
Clinical Diagnosis : Left breast cancer
Nature of Specimen : Referred blocks labelled as 7014/24 & 1416/24 [US guided biopsy]
Receiving Date : 2024-11-24
Patient ID : 679248

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections prepared from the paraffin block were stained for HER 2 gene and chromosome 17 using Silver In Situ Hybridization (SISH) technique in Benchmark XT (Ventana).

Results :

The ratio of Her 2 signals to chromosome 17 signals in the nuclei of the tumour cells is 2.5 (more than 2) indicating amplification of HER2 gene.
Average Her 2 copy nb/cell is 4.7.

Conclusion :

Referred blocks, HER 2 by SISH, POSITIVE HER 2 GENE AMPLIFICATION, RATIO IS 2.5.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

CamScanner
Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name:	Klear Nabil Fawzy	Patient ID:	701872
Prof. Dr:	Emad Mohsen Barsoom	Exam Date:	25/11/2024
Accession:	241124043		

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

Female patient 38 years old, Weight: 82 Kg, Height: 155 cm, Blood Glucose level: 96 mg/dl

Radiopharmaceutical:-

F-18-FDG 7.5 mCi injected I.V.

Clinical background:-

- Patient presented with left breast mass, pathologically proven invasive duct carcinoma grade III.

Purpose of the study:-

- Detection of any neoplastic lesions.

Technique:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~ 2.78.
- Mediastinal blood pool average SUV ~ 1.6.

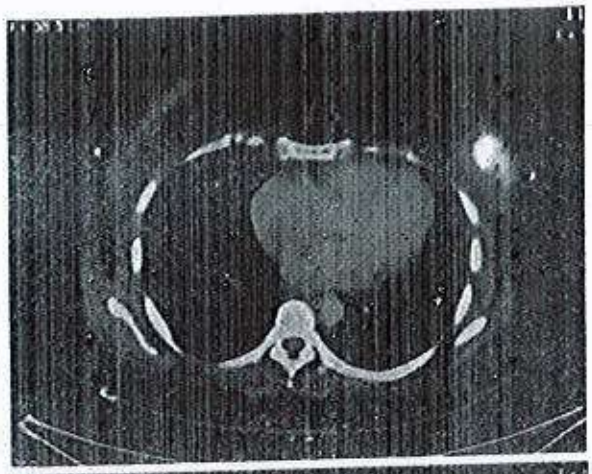
CamScanner

PET Scan
[[DOCTYPE:PET_SCAN]]

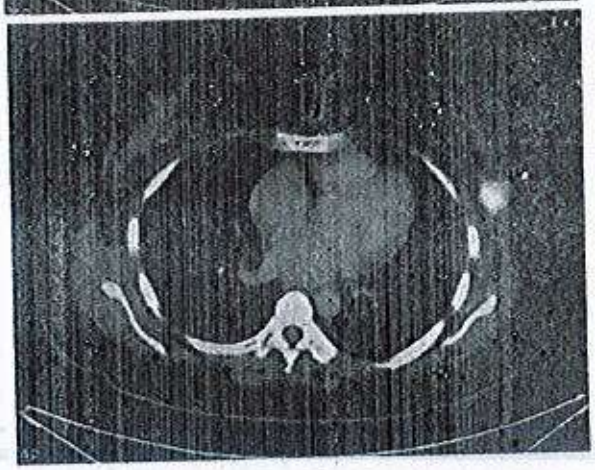
Patient Name:	*Klear Nabil Fawzy*	*Patient ID:*	*701872*
Prof. Dr:	*Emad Mohsen Barsoom*	*Exam Date:*	*25/11/2024*
Accession:	*241124043*		

Findings:-

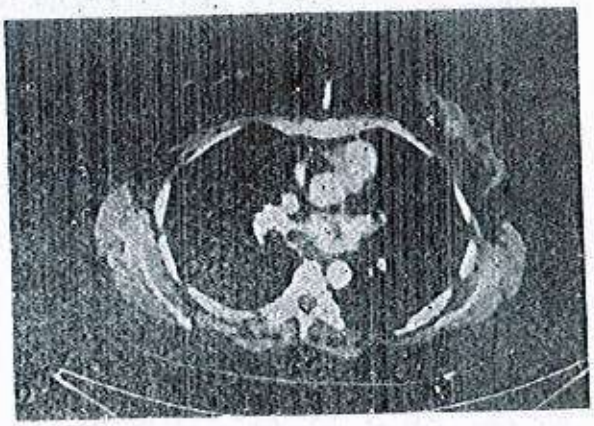
- ***Breast and axillae***
  - Metabolically active left breast upper outer quadrant confluent masses and nodules are seen measuring collectively about 2.5 x 4 x 2.4 cm along its maximum AP, TS and CC dimensions with max SUV ~ 12.14.



  - Smaller nodule is noted at lingular level measuring 2.5 x 2.5 cm with max SUV ~ 7.18.
  - Small left breast upper central low grade metabolically active nodule is seen measuring 1.9 cm with max SUV ~ 2.29.



  - Few low grade metabolically active left axillary and left retro pectoral lymph nodes are seen measuring up to 2 cm with max SUV ~ 1.3 (retro pectoral).
  - No metabolically active right breast masses or axillary lymph nodes.
  - Few metabolically inactive (non FDG avid) left axillary lymph nodes are seen measuring up to 1.7 cm (inflammatory).



CamScanner

PET Scan
[[DOCTYPE:PET_SCAN]]